1. A 60-years old patient presented with manifestation of chronic kidney disease for dialysis; he had a family history of hemodialysis (his father and one of his brothers).What do you think the diagnosis
A) polycystickidneydisease
B) nephroticsyndrome
C) chronicNSAIDsabuse
D) interstitialnephritis

2. After the AED has delivered the shock you should
A) Wait patiently for the AED to reanalyze
B) Give 2 breaths
C) Continue CPR beginning with compressions
D) Turn off the AED

3. urgency due to decreased bladder capacity is associated with
A) Urethritis
B) Cystitis
C) Pregnancy
D) Urethral stones

4. A 46-year-old women comes to urology assessment clinic for review. She is complaining of incontinent urine with cough or sneeze, her complaint is:
A) Urge incontinence
B) Stress incontinence
C) Retention with overflow incontinence
D) Total incontinence

5. What is the correct rate of compressions?
A) 60
B) 80
C) 100
D) 40

6. causes of over-active bladder includes:
A) Urethritis
B) Cystitis
C) Pregnancy

D) Idiopathic

7. You are alone and find a potential victim. What is your first step?
A) Check for responsiveness
B) Ensure scene safety
C) Begin Chest Compressions
D) Open the airway and give 2 breaths

8. You have noticed a potential victim and ensured the scene is safe. What is your next step
A) Check for victim's responsiveness
B) Begin chest compressions
C) Provide rescue breathing
D) Call the victim's doctor

9. During CPR on an adult victim, the compression-to-breath ratio is:
A) Give 3 Breaths after every 15 compressions
B) Give 2 breaths after every 30 compressions
C) Give 1 breath after every 5 compressions
D) Give 2 breaths after every 100 compressions

10. Before providing rescue breathing for an unresponsive victim, you must check for breathing. You do this by listening and feeling for airflow through the victim's nose or mouth and by
A) Looking into the victim's mouth to see if anything is blocking the airway
B) Shaking or tapping the victim's shoulder to stimulate him to breathe
C) Checking the pupils
D) Looking to see if the chest rises (and falls) as the victim breathes

12.A patient presented with manifestation of chronic kidney disease for dialysis ;he had a very long history of disc prolapse and sciatica. What do you think the accused drug?
A) NSAIDs
B) Steroid
C) Antibiotics
D) Alpha-blocker

13. Bladder outlet obstruction may be caused by
A) Prostate hypertrophy

B) Ureteral stones
C) Pregnancy
D) Sexually transmitted disease

14.Deep palpation is used to?
A) Detect small bowel movements
B) Identify surface irregularities
C) Assess organ size and deep masses
D) Feel the pulsations of the aorta

15. How is an enlarged spleen best palpated?
A) Using deep palpation in the right lower quadrant
B) Using light palpation in the left upper quadrant
C) With deep palpation in the left upper quadrant while the patient takes a deep breath
D) By percussing over the stomach

16. In which condition is a positive Carnett s sign observed?
A) Gastric ulcer
B) Abdominal wall pain
C) Peritonitis
D) Pancreatitis

17. visible peristalsis in an adult may suggest:
A) Normal digestion
B) Intestinal obstruction
C) Healthy bowel motility
D) Dehydration

18. A scaphoid abdomen may be seen in:
A) Obesity
B) Ascites
C) Severe malnutrition
D) Pregnanc

19. Caput medusae (dilated veins around the umbilicus) is a sign of:
A) Portal hypertension
B) Appendicitis

C) Pancreatitis
D) Diverticulitis

20. Light palpation of the abdomen is performed to assess:
A) Liver size
B) Tenderness and guarding
C) Kidney enlargement
D) Aortic pulsation

21. Rebound tenderness (Blumberg's sign) is indicative of:
A) Cholecystitis
B) Peritonitis
C) Gastritis
D) Hepatitis

22. Murphy's sign is used to diagnose:
A) Appendicitis
B) Acute cholecystitis
C) Pancreatitis
D) Diverticulitis

23. A palpable, pulsatile abdominal mass suggests:
A) Hepatomegaly
B) Abdominal aortic aneurysm
C) Splenomegaly
D) Renal cyst

24. The spleen is best palpated by:
A) Deep palpation in the right upper quadrant
B) Approaching from the right lower quadrant
C) Asking the patient to take a deep breath while palpating the left upper quadrant
D) Percussing Traube's space

25. Rovsing's sign (right lower quadrant pain upon left-sided palpation) is seen in:
A) Cholecystitis
B) Appendicitis
C) Pancreatitis

D) Gastric ulcer

26. Percussion of the abdomen is used to assess:
A) Bowel sounds
B) Organ size and presence of fluid/gas
C) Tenderness
D) Muscle rigidity

27. Shifting dullness on percussion suggests:
A) Pneumoperitoneum
B) Ascites
C) Hepatomegaly
D) Ileus

28. Hyper resonance on abdominal percussion is most indicative of:
A) Ascites
B) A large ovarian cyst
C) Intestinal obstruction with gas distension
D) Hepatomegaly

29. The normal liver span is:
A) 2-4 cm at the midclavicular line
B) 6-12 cm at the midclavicular line
C) 15-20 cm at the midclavicular line
D) 20-25 cm at the midclavicular line

30. A tympanic note on percussion is most common over the:
A) Liver
B) Stomach or intestines
C) Spleen
D) Kidneys

31. Auscultation of the abdomen should be performed:
A) Before palpation and percussion
B) After palpation and percussion
C) Only if pain is reported
D) Only in suspected bowel obstruction

32. Normal bowel sounds occur approximately:
A) Every 1-2 minutes
B) Every 5-10 seconds
C) . Every 30-60 seconds
D) Only after meals

33.Hyperactive bowel sounds (borborygmi) are associated with:
A) Paralytic ileus
B) Early intestinal obstruction
C) Peritonitis
D) Ascites

34. Absent bowel sounds for >2 minutes suggest:
A) Normal digestion
B) Paralytic ileus or peritonitis
C) Gastroenteritis
D) Constipation

35. A bruit heard over the renal arteries may indicate:
A) Normal blood flow
B) Renal artery stenosis
C) Appendicitis
D) Gastric ulcer

36. Cullen’s sign (periumbilical ecchymosis) is seen in:
A) Acute pancreatitis
B) Appendicitis
C) Diverticulitis
D) Hepatitis

37. Grey Turner’s sign (flank ecchymosis) is associated with:
A) Ruptured ectopic pregnancy
B) Severe pancreatitis or retroperitoneal hemorrhage
C) Cholecystitis
D) Gastric perforation

38. The psoas sign (pain on hip extension) suggests:
A) Cholecystitis
B) Appendicitis
C) Diverticulitis
D) Splenomegaly

39. Obturator sign (pain on internal hip rotation) is seen in:
A) Pelvic appendicitis
B) Cholelithiasis
C) Pancreatitis
D) Hepatitis

40. Kehr's sign (left shoulder pain) is associated with:
A) Splenic rupture
B) Renal colic
C) Gastric ulcer
D) Liver abscess

41. A rigid, board-like abdomen is most concerning for:
A) Constipation
B) Peritonitis
C) Gastroenteritis
D) Irritable bowel syndrome

42. Hepatomegaly is best assessed by:
A) Deep palpation and percussion
B) Auscultation alone
C) Inspection for jaundice
D) Rebound tenderness

43.Splenomegaly typically expands toward the:
A) Right upper quadrant
B) Left lower quadrant
C) Right lower quadrant
D) Epigastric region

44. A succussion splash heard >2 hours after eating suggests:

A) Normal gastric emptying
B) Gastric outlet obstruction
C) Healthy peristalsis
D) Diverticulosis

45. Light palpation is primarily used to assess?
A) Deep organ masses
B) Surface tenderness, muscle guarding, and rigidity
C) Bowel sounds
D) Organ enlargement

46. Normal subcostal angle is?
A) 90
B) 100
C) 110
D) 120

47. Pathological Findings and Special Tests Murphy s sign is positive in which condition?
A) Appendicitis
B) Cholecystitis
C) Gastritis
D) Pancreatitis

48. positive Rovsing s sign suggests?
A) Cholecystitis
B) Peritonitis
C) Appendicitis
D) Bowel obstruction

49. which of the following is a common cause of heart burn?
A) Peptic ulcer disease
B) Gastroesophageal reflux disease (GERD)
C) Gallbladder disease
D) Gastric cancer

50. Which of the following findings could be an indication of intra abdominal bleeding during abdominal inspection ?

A) Tenderness around the umbilicus
B) Abdominal rigidity and gurding
C) Ecchymosis around the flanks (Grey Turners sign)
D) Anorexia

51. What is the significance of abdominal asymmetry ?
A) It may indicate a mass or localized swelling
B) Its anormal finding
C) It indicates hernia
D) Done of the above

52. The presence of an abdominal scar may indicated :
A) Previous surgery
B) A hernia
C) Inflammatory diseases
D) All of the above

53. What condition may present with a visible distended abdomen and a tympanic sound on percussion ?
A. Ascites
B. Intestinal obstruction
C. Cirrhosis
D. Pancreatitis

54. In abdominal inspection , a normal finding is:
A) A. Visible veins
B) B. Significant skin changes
C) C. A flat or slightly rounded contour
D) Visible perstalsis

55. Which condition can cause the abdomen to appear " rock- rigid" or rigid on palpation ?
A) A. Liver cirrhosis
B) B. Pancreatitis
C) C. Peritonitis
D) D. Cholicystitis

56. The appearance of the abdomen in a patient with liver cirrhosis is often characterised by :

A. A bulging and smooth appearance
B. Yellowing of the skin
C. Visible veins and ascites
D. Abnormal percussion sounds

57. The presence of a single large abdominal mass could suggest :
A. Liver tumor
B. Pancreatic cyst
C. Ovarian mass
D. All of the above

58. During the abdominal inspection what might a " fluid wave " indicate?
A. Ascites
B. Pregnancy
C. Gastroenteritis
D. Gastric reflux

59. What is the recommeded patient for abdominal inspection ?
A. Supine with knee extended
B. Supine with knee flexed
C. Prone
D. Standing with arms raised

60. What is the best position for the patient for during an abdominal inspection ?
A) Sitting upright
B) Supine with arms folded across the chest
C) Supine with arms at the sides and knees slightly flexed
D) Prone position

61. What is caput medosa ?
A) Dilated veins around the umbilicus
B) A visible pulsation in the abdomen
C) A type of abdominal distension
D) A form of abmormal abdominal skin pigmentation

62. What is the normal shape of the abdomin in a healthy adult
A) Scaphoid

B) distended
C) Flate or slightly rounded
D) Protuberant

63. A scaphoid abdomen is commonaly seen in :
A) Obesity
B) Malnutrition
C) Ascites
D) Pregnancy

64. Which of the following can cause generalized abdominal distension ?
A) Ascites
B) Obesity
C) Intestinal obstruction
D) All of the above

65. The presence of dilated veins around the umbilicus (ccaput medosa) is indicative of :
A) Appendicites
B) Cirrhosis with portal hypertension
C) Pancreatitis
D) Gastric cancer

66. Striae (stretch marks) is Cushings syndrome are typically :
A) White and narrow
B) Pink and short
C) Purple and wide
D) Absent

67. A visible pulsation in the epigastric region may indicates :
A) Ascites
B) Aortic aneurysm
C) Umbilical hernia
D) Intestinal obstruction

68. What can cause a pulsatile mass or visible pulsation in the epigastrium ?
A) Hepatitis
B) Abdominal aortic aneurysm

C) Ascites
D)Pancreatitis

69. Specific Abdominal Regions and Structures Which quadrant is palpated first during an abdominal exam?
A) The quadrant with the most pain
B) The lower right quadrant
C) The upper left quadrant
D) The quadrant opposite the area of pain

70. The iliopsoas test is performed to assess?
A) Gastric ulcers
B) Appendicitis
C) Liver enlargement
D) Small bowel obstruction

71. The liver is best palpated in which region?
A) Right upper quadrant (RUQ)
B) Left upper quadrant (LUQ)
C) Right lower quadrant (RLQ)
D) Left lower quadrant (LLQ)

71. The presence of a positive Grey Turner s sign suggests?
A) Liver cirrhosis
B) Retroperitoneal hemorrhage
C) Appendicitis
D) Cholecystitis

72. The rebound tenderness test (Blumberg's sign) is used to detect?
A) Liver enlargement
B) Appendicitis or peritoneal irritation
C) Aortic aneurysm
D) Gallbladder disease

73. What does shifting dullness on percussion indicate?
A) Splenomegaly
B) Bowel obstruction

C) Ascites
D) Pancreatitis

74. What is a classic finding of peritonitis?
A) Soft, non-tender abdomen
B) Involuntary guarding and rebound tenderness
C) Increased bowel sounds
D) Pulsatile mass

75. What is the normal finding when palpating the liver?
A) Soft, smooth, non-tender edge below the right costal margin
B) Hard, irregular, and nodular
C) Painful and pulsating
D) Not palpable at all

76. Which condition is suggested by a palpable, pulsatile mass in the mid-abdomen?
A) Liver cirrhosis
B) Aortic aneurysm
C) Gastritis
D) Gallstones

76. Which of the following is the best position for the patient during abdominal palpation?
A) Standing
B) Sitting upright
C) Supine with knees slightly flexed
D) Prone

77. Which of the following is the most likely cause of a rigid abdomen?
A) Peritonitis
B) Irritable bowel syndrome
C) Gastric ulcer
D) Cholelithiasis

78. A patient asks, "Do you think I need an X-ray?" This question is BEST categorized as which part of ICE?
A) Concern
B) Expectation

C) Idea
D) Presenting Complaint

79.According to the text, what is the BEST way to initiate gathering information about a patient's presenting problem?
A) Ask direct, specific questions about potential diagnoses.
B) Begin by reviewing the patient's past medical history not the current complaint.
C) Start with a closed question to quickly identify the main issue.
D) Use an open question to encourage the patient to describe their problem by own way.

80. Asking a patient with abdominal pain about nausea, vomiting, or diarrhea, what these symptoms MOST relevant to?
A) Hospital Course
B) Inquiry about other symptoms of the same system
C) Past Medical History
D) Systemic Review

81. Asking about bowel habits (frequency, consistency, changes) falls under which category in complaint analysis?
A) Cardiovascular System Inquiry
B) Gastrointestinal System Inquiry
C) Genitourinary System Inquiry
D) Respiratory System Inquiry

82. Concerning radiation of pain, what does this term describe?
A) How intense the pain is?
B) The quality of the pain sensation
C) What makes the pain worsen?
D) Whether the pain moves to other areas

83. For a patient with a primary complaint of headache, asking about vision changes or weakness for which one of the following this belongs?
A) Inquiry about other symptoms of the same system
B) Irrelevant to the chief complaint
C) Part of pain character assessment
D) Part of the systemic review

84. If there are more than one complaint, by which manner you will arrange when your write your documents?
A) Arrange them according to patient`s preference
B) Arrange them according to yours preference
C) Arrange them chronologically from the old to the recent.
D) Arrange them chronologically from the recent to the old.

85. Regarding aggravating factors, what are you trying to identify?
A) Activities that worsen the pain
B) Emotions that cause the pain
C) Things that make the pain better
D) Underlying medical conditions

86. Regarding the course of pain, which term describes pain that comes and goes?
A) Chronic
B) Constant
C) Intermittent
D) Radiating

87.To understand the duration of pain, which question is helpful?
A) How long the pain episode lasts
B) If the pain is affected by weather
C) What time of day the pain occurs
D) When you begins to experience the pain

88. When asking about the onset of pain, which question is MOST relevant?
A) "Can you describe what the pain feels like?"
B) "Did the pain start suddenly or gradually?"
C) "Does anything make the pain better?"
D) "Where exactly does it hurt?"

89.When asking about the site of pain, which approach is BEST?
A) Asking the patient to point to the area that hurts most
B) Assuming the pain is generalized unless stated otherwise
C) Guessing where the pain is located based on the complaint
D) Ignoring the location and focusing on pain character

90. When performing a systemic review, if a patient reports night sweats, this is MOST relevant to which system?
A) General/Constitutional
B) Musculoskeletal
C) Neurological
D) Respiratory

91. Which component of the 'History of Present Illness' is MOST helpful in tracking the progression of a patient's condition while admitted?
A) Analysis of presenting Complain
B) Hospital Course
C) Inquiry about other symptoms of the same system
D) Systemic Review

92. Which element of the 'History of Present Illness' checklist helps to broaden the diagnostic possibilities beyond the initial complaint?
A) Hospital Course
B) Inquiry about other symptoms of the same system
C) Inquiry about the presenting Complaint
D) Systemic Review “Symptoms of other systems”

93. Which of the following statement is most accurate about severity of pain?
A) Documenting pain for legal purposes
B) Guiding treatment decisions and monitoring progress
C) It`s objective and can rely on it solely
D) It`s subjective and can`t rely on it solely

94. Which question BEST explores radiation of back pain?
A) "Does the pain stay in your back or move anywhere else?"
B) "How long have you had this back pain?"
C) "Is your back pain mild, moderate, or severe?"
D) "What makes your back pain worse?"

95. Which question BEST explores relieving factors for joint pain?
A) "Did your joint pain start suddenly or gradually?"
B) "Does rest, heat, or ice help your joint pain?"
C) "What does your joint pain feel like?"

D) "Where is the joint pain located?"

96. 40 years old gentleman complaining of abdominal swelling , during abdominal palpation cystic mass is founded at right lumber area , the most likely diagnosis is?
A) Polycystic kidney
B) Renal carcinoma
C) Renal infarction
D) Renal stone

97. 42 years lady, her abdominal examination shows pigmentations at flanks , this sign is?
A) Virchow s sign
B) Murphy s sign
C) Cullen s sign
D) Grey Turner s sign

98. 45 years lady complaining of abdominal swelling , During her abdominal examination it was noticed that the swelling was reduced in size while raising up her shoulders in supine position , the most likely diagnosis is?
A) Pancreatic mass
B) Abdominal wall lipoma
C) Rectus sheath hematoma
D) Para Umbilical hernia

99. 72-years-old patient During abdominal palpation , the liver is tender in?
A) Hemangioma
B) Hepatic cancer
C) Liver cirrhosis
D) Bilharziasis

100. Acute cholecystitis is diagnosed if the patient has positive?
A) Virchow s sign
B) Murphy s sign
C) Cullen s sign
D) Grey Turner s sign

101. During abdominal examination for acute appendicitis , we should start palpation from?
A) Right hypochondrial area

B) Left hypochondrial area
C) Right iliac fossa
D) Left iliac fossa

102. Normal span of the left lobe of liver is?
A) 1 - 4 cm
B) 4 - 8 cm
C) 8 - 12 cm
D) 12 16 cm

103. The best patient s position for doing PR is?
A) Supine
B) Prone
C) Left lateral
D) Right lateral

104. The renal angle lies between?
A) Rectus muscle and last rib
B) Rectus muscle and inguinal ligament
C) Sacrospinalis muscle and last rib
D) Sacrospinalis muscle and inguinal ligament

105. Thrill is a sign of?
A) Hemangioma
B) Hernia
C) Aneurysm
D) Lipoma

106. A tympanic sound on abdominal percussion usually indicates:
A) Solid organ
B) Fluid accumulation
C) Gas-filled structure
D) Bone

107. Fluid thrill and shifting dullness are signs of:
A) Gastric outlet obstruction
B) Hepatocellular carcinoma

C) Peritonitis
D) Ascites

108. In a patient with intestinal obstruction, percussion may reveal:
A) Dullness
B) Hyperresonance
C) Tympany
D) Resonance

109. In splenomegaly, percussion over the left lower anterior chest may reveal:
A) Increased tympany
B) Dullness
C) Resonance
D) No sound

110. Percussion can help distinguish between:
A) Inflammatory vs. non-inflammatory disease
B) Intraperitoneal vs. retroperitoneal organs
C) Painful vs. painless conditions
D) Acute vs. chronic infection

111. Percussion helps differentiate between:
A) Acute vs. chronic pain
B) Solid vs. gas or fluid
C) Inflammation vs. infection
D) Somatic vs. visceral pain

112. Percussion is least helpful in identifying:
A) Liver span
B) Spleen size
C) Pancreas
D) Ascites

113. Percussion over a fluid-filled abdomen typically yields a:
A) Tympanic note
B) Resonant note
C) Dull note

D) Hyperresonant note

114. Percussion over the bladder is useful when:
A) The patient has a fever
B) Bladder is empty
C) Bladder is full
D) There is hematuria

115. Shifting dullness on percussion is used to detect:
A) Constipation
B) Ascites
C) Peritonitis
D) Peptic ulcer

120. The best position to elicit shifting dullness is:
A) Supine
B) Sitting
C) Left lateral decubitus
D) Standing

121. The technique of percussion involves:
A) Auscultating for bowel sounds
B) Striking one finger with another
C) Palpating for tenderness
D) Measuring abdominal girth

122. Traube's space is used in percussion to evaluate:
A) Hepatomegaly
B) Gastritis
C) Splenomegaly
D) Appendicitis

123. Tympanic sounds on percussion generally indicate:
A) Air-filled viscera
B) Inflammation
C) Hemorrhage
D) Tumor

124. What does a board-like rigidity with generalized abdominal dullness suggest?
A) Normal finding
B) Ascites
C) Peritonitis
D) Intestinal obstruction

125. What does percussion over Traube's space usually produce?
A) Dullness
B) Tympany
C) Hyperresonance
D) Flatness

126. What is the primary purpose of percussion in the GIT examination?
A) Assess muscle strength
B) Detect organ enlargement
C) Measure pulse rate
D) Identify bowel sounds

127. What percussion note is heard over a feces-filled colon?
A) Tympanic
B) Dull
C) Hyperresonant
D) Flat

128. What sign uses percussion to detect splenic enlargement?
A) Grey-Turner's sign
B) Castel sign
C) Cullen's sign
D) Kehr's sign

129. When percussing for fluid in the abdomen, which finding indicates tense ascites?
A) Constant tympany
B) Shifting area of dullness
C) Fixed dullness
D) Hyperresonance

130. When percussing the liver span, where is dullness first typically noted?
A) Right 5th intercostal space
B) Left 5th intercostal space
C) Umbilicus
D) Right iliac fossa

131. Where is the spleen normally not percussible?
A) Posterior axillary line
B) Midclavicular line
C) Anterior axillary line
D) Traube's space

132. Which is a normal finding during percussion of the abdomen?
A) Generalized dullness
B) Localized tympany in stomach region
C) Hyperresonance in all quadrants
D) No sound heard

133. Which method confirms free fluid in the peritoneal cavity?
A) Murphy's sign
B) Shifting dullness
C) McBurney's point tenderness
D) Psoas sign

134. Which of the following areas is normally dull on abdominal percussion?
A) Stomach
B) Liver
C) Colon
D) Small intestine

135. Which of the following best describes a normal percussion finding in the left upper quadrant?
A) Dullness over the spleen
B) Tympany from stomach gas
C) Resonance from lungs
D) Hyperresonance from bowel

136. Which organ is percussed in the right upper quadrant to detect hepatomegaly?
A) Stomach
B) Spleen
C) Liver
D) Kidney

137. Which percussion note is expected over a normal stomach?
A) Resonant
B) Hyperresonant
C) Tympanic
D) Dull

138. Which percussion note would suggest abdominal mass with no gas?
A) Tympanic
B) Dull
C) Hyperresonant
D) Resonant

140. A history of contrast exposure is important for?
A) Contrast-induced nephropathy
B) Interstitial nephritis
C) UTI
D) Nephrotic syndrome

141. A patient presents with frothy urine. This is indicative of ?
A) Proteinuria
B) Hematuria
C) Glycosuria
D) Pyuria

142. A positive family history is most relevant in which renal condition?
A) UTI
B) Acute kidney injury
C) Polycystic kidney disease
D) Glomerulonephritis

143. A toddler with poor weight gain and anemia should be evaluated for?

A) UTIs
B) CKD
C) Diabetes
D) Appendicitis

144. Anuria refers to?
A) Urine output less than 50 mL/day
B) No urine output at all
C) Excessive urination
D) Pus in urine

145. Asking about analgesic use is important to identify risk of?
A) Renal calculi
B) Analgesic nephropathy
C) Nephrotic syndrome
D) Cystitis

146. Asking about foamy urine helps assess?
A) Hematuria
B) Pyuria
C) Proteinuria
D) Glucosuria

147. Asking about nocturnal enuresis is relevant in?
A) Diabetes
B) UTIs
C) Pediatric renal disease
D) All of the above

148. Asking about systemic diseases is important because?
A) They may affect the bladder
B) They may cause secondary renal disease
C) They mimic renal disease
D) All of the above

149. Asking about travel history helps identify?
A) Malaria-related nephropathy

B) Hydronephrosis
C) Diabetes
D) Appendicitis

150. Bilateral pedal edema in renal patients suggests?
A) Ureteral obstruction
B) Nephrotic syndrome
C) Renal artery stenosis
D) Acute pyelonephritis

151. Chronic NSAID use most commonly affects which part of the kidney?
A) Glomerulus
B) Tubules
C) Collecting ducts
D) Renal artery

152. Dark cola-colored urine suggests?
A) Lower tract hematuria
B) Myoglobinuria
C) Glomerular hematuria
D) Bacterial UTI

153. Diabetes may cause which type of kidney disease?
A) Glomerulonephritis
B) Nephrosclerosis
C) Diabetic nephropathy
D) Pyelonephritis

154. Enuresis in a 7-year-old should prompt evaluation for?
A) Glomerulonephritis
B) UTI
C) Diabetes insipidus
D) Polycystic kidney disease

155. Family history is least likely to be relevant in?
A) Polycystic kidney disease
B) Alport syndrome

C) Renal stones
D) UTI

156. Flank pain radiating to the groin suggests?
A) Appendicitis
B) Renal colic
C) Bowel obstruction
D) Cystitis

157. Gross hematuria usually indicates?
A) Bladder source
B) Glomerular source
C) Liver disease
D) Prostatic hypertrophy

158. Hematuria after exercise typically indicates?
A) Tumor
B) Trauma
C) Benign hematuria
D) Glomerulonephritis

159. Hematuria associated with pain is most likely due to?
A) Glomerulonephritis
B) Bladder cancer
C) Kidney stones
D) Diabetic nephropathy

160. History of prolonged obstructive symptoms may lead to?
A) Hydronephrosis
B) Renal calculi
C) Bladder atony
D) All of the above

161. History of rapid weight gain in a renal patient is usually due to?
A) Muscle hypertrophy
B) Fluid retention
C) Increased fat mass

D) Constipation

162. Inquiring about recent sore throat is relevant because it may precede?
A) Interstitial nephritis
B) Post-streptococcal glomerulonephritis
C) Renal artery stenosis
D) Acute tubular necrosis

163. Microscopic hematuria with RBC casts suggests?
A) Bladder cancer
B) Nephritic syndrome
C) Ureteral stone
D) Prostatitis

164. NSAID overuse may lead to?
A) Polycystic kidneys
B) Interstitial nephritis
C) Glomerulonephritis
D) Renal artery stenosis

165. Oliguria is defined as urine output less than?
A) 1 L/day
B) 500 mL/day
C) 250 mL/day
D) 1000 mL/day

166. Past exposure to heavy metals is relevant to assess?
A) Acute cystitis
B) Chronic interstitial nephritis
C) Urinary incontinence
D) Hypertension

167. Poor appetite in a renal patient may suggest?
A) UTI
B) Acute renal failure
C) CKD
D) Nephrolithiasis

168. Post-coital dysuria in females may suggest?
A) Glomerulonephritis
B) Urethral syndrome
C) Nephrolithiasis
D) Interstitial cystitis

169. Pruritus in a renal history may suggest?
A) CKD
B) UTI
C) Glomerulonephritis
D) Diabetes

170. Recurrent UTIs may lead to?
A) Nephrolithiasis
B) CKD
C) Nephritic syndrome
D) All of the above

171. Sexual history is important when evaluating?
A) Urethritis
B) Renal artery stenosis
C) Nephrotic syndrome
D) CKD

172. Smoking history is relevant in renal history due to its link with?
A) UTIs
B) Renal cell carcinoma
C) Nephrotic syndrome
D) Nephritic syndrome

173. Sudden severe loin pain and hematuria may indicate ?
A) Nephrotic syndrome
B) UTI
C) Renal colic
D) CKD

174. What is the most important lifestyle history in renal stone formation?
A) Smoking
B) Water intake
C) Alcohol consumption
D) Exercise frequency

175. What is the term for painful urination?
A) Nocturia
B) Dysuria
C) Polyuria
D) Hematuria

176. What symptom may indicate renal disease in elderly patients?
A) Memory loss
B) Generalized weakness
C) Persistent cough
D) Chest pain

177. Which comorbidity is a leading cause of chronic kidney disease (CKD)?
A) Asthma
B) Diabetes mellitus
C) Hyperthyroidism
D) Depression

178. Which history component is key for evaluating obstructive uropathy?
A) Family history
B) Bowel habits
C) Voiding pattern
D) Past infections

179. Which history detail is most important for early CKD detection?
A) Urinary frequency
B) Serum creatinine
C) Family history
D) Routine screening

180. Which of the following drugs is most likely to cause nephrotoxicity?

A) Acetaminophen
B) Ibuprofen
C) Amoxicillin
D) Metformin

181. Which of the following is a lower urinary tract symptom?
A) Flank pain
B) Hesitancy
C) Fever with chills
D) Costovertebral angle tenderness

182. Which of the following is a red flag symptom in renal history?
A) Mild backache
B) Polyuria
C) Painless hematuria
D) Increased thirst

183. Which of the following is often silent in early stages in elderly patients?
A) UTI
B) Nephrotic syndrome
C) CKD
D) Renal artery stenosis

184. Which of the following is the most common presenting symptom of renal disease?
A) Headache
B) Hematuria
C) Chest pain
D) Dizziness

185. Which symptom indicates the presence of nephrotic syndrome?
A) Hematuria
B) Edema
C) Flank pain
D) Fever

186. Which symptom is least likely to be associated with chronic kidney disease?
A) Fatigue

B) Polyuria
C) Pruritus
D) Sudden severe back pain

187. Which symptom is more common in children with nephrotic syndrome?
A) Hematuria
B) Edema
C) Vomiting
D) Abdominal pain

188. Which symptom suggests urinary tract infection (UTI)?
A) Polyuria without fever
B) Suprapubic pain with dysuria
C) Flank pain without urinary symptoms
D) Nocturia only

189. White blood cell casts in urine are indicative of?
A) Pyelonephritis
B) Nephrotic syndrome
C) Renal artery stenosis
D) Glomerulonephritis